

Timazol Tablet 5mg

CONTENT

Each tablet contains Methimazole 5mg.

DESCRIPTION

White round biconvex tablet embossed with "S03" on one side and a breakline on the other.

PROPERTIES

Pharmacodynamics

Methimazole (also known as thiamazole) is an antithyroid drug that acts by blocking the production of thyroid hormones through 2 mechanisms i.e. by interfering with the incorporation of iodine into tyrosyl residues of thyroglobulin; and also by inhibiting the coupling of these iodotyrosyl residues to form iodothyronine.

Methimazole does not inhibit the action of thyroid hormones that are already formed and present in the thyroid gland or circulation, nor does the drug interfere with the effectiveness of exogenously administered thyroid hormones. Methimazole is at least 10 times more potent than propylthiouracil on a weight basis.

Pharmacokinetics

The pharmacokinetics of methimazole can be considered together with those of carbimazole since carbimazole is rapidly and completely metabolised to methimazole in the body.

Following oral administration, methimazole is rapidly absorbed from the gastrointestinal tract with peak plasma concentrations occurring within about 30 to 60 minutes. It is metabolised rapidly and the elimination half-life ranges from 5 – 13 hours. Methimazole is excreted in urine.

INDICATION

Timazol is indicated in the treatment of hyperthyroidism. Long-term therapy may lead to remission of the disease. **Timazol** may also be used to ameliorate hyperthyroidism in preparation for subtotal thyroidectomy or radioactive iodine therapy.

RECOMMENDED DOSAGE

Timazol is administered orally and daily dosage is usually given in 3 divided doses at approximately 8-hour intervals.

Adult:

- Mild hyperthyroidism: 3 tablets (15mg) daily in 3 divided doses.
- Moderately severe hyperthyroidism: 6 – 8 tablets (30 – 40mg) daily in 3 divided doses.
- Severe hyperthyroidism: 12 tablets (60mg) daily in 3 divided doses.
- The maintenance dose is 1 – 3 tablets (5 to 15mg) daily.

Children:

- The initial dose is 0.4 mg/kg body weight divided into 3 doses.
- The maintenance dose is half of the initial dose.

CONTRAINDICATIONS

- Patients with hypersensitivity to methimazole or to any of the excipients.
- Nursing mothers – as the drug is excreted in breast milk.
- Patients with a history of acute pancreatitis after administration of carbimazole or active metabolite, methimazole (thiamazole).

WARNINGS / PRECAUTIONS

Methimazole may cause white cell disorders such as neutropenia and agranulocytosis, which may be fatal if treatment with methimazole is not stopped promptly. These reactions usually occur during the first 3 months of therapy, and in most cases, are reversible on stopping treatment. Since agranulocytosis can develop very rapidly, periodic leucocyte counts alone may not be effective in the early detection of these reactions.

Patients who receive methimazole should be under close surveillance. Physicians should encourage patients to immediately report any evidence of illness or unusual clinical symptoms, particularly sore throat, skin eruptions, fever, headache or general malaise. In such cases, white blood cell and differential counts should be made to determine whether agranulocytosis has developed. Particular care should be exercised with patients who are receiving additional drugs known to cause agranulocytosis.

The development of arthralgias should prompt drug discontinuation, since this symptom may indicate a severe transient migratory polyarthrititis known as "the antithyroid arthritis syndrome".

Treating patients with both methimazole and warfarin necessitates intensive and frequent monitoring, in particular when initiating, discontinuing or changing doses of methimazole, since alterations in the thyroid function affect the response to anticoagulation.

Methimazole can cause hypothyroidism necessitating routine monitoring of TSH and free T4 levels with adjustments in dosing to maintain a euthyroid state. Because the drug readily crosses placental membranes, methimazole can cause foetal goiter and cretinism when administered to a pregnant woman. For this reason, it is important that a sufficient, but not excessive, dose be given during pregnancy.

Hepatotoxicity is a rare adverse reaction in patients treated with methimazole. Although there have been reports of hepatotoxicity (including acute liver failure) associated with methimazole, the risk of hepatotoxicity appears to be less with methimazole than with propylthiouracil, especially in the paediatric population. There have been rare reports of fulminant hepatitis, hepatic necrosis, encephalopathy and death. Cholestatic jaundice has occurred rarely. Patients should be instructed to report symptoms of hepatic dysfunction such as jaundice, anorexia, pruritus, and/or right upper-quadrant pain. Their presence should prompt evaluation of liver function tests and discontinuation of methimazole. Drug treatment should be discontinued promptly in the event of clinically significant evidence of liver abnormality, including hepatic transaminase values exceeding 3 times the upper limit of normal.

Because methimazole may cause hypoprothrombinaemia and bleeding, prothrombin time should be monitored during therapy with the drug, especially before surgical procedures. Thyroid function tests should be monitored periodically during therapy. Once clinical evidence of hyperthyroidism has resolved, the finding of a rising serum TSH indicates that a lower maintenance dose of methimazole should be employed.

Cases of vasculitis have been observed very rarely in patients receiving methimazole therapy. The cases of vasculitis include: leukocytoclastic cutaneous vasculitis, glomerulonephritis, and systemic vasculitis (with fatal outcome). Many cases were associated with anti-neutrophilic cytoplasmic antibodies (ANCA)-positive vasculitis. Early recognition of vasculitis is important to prevent long-term organ damage and/or death. Inform patients to promptly report symptoms that may be associated with vasculitis including rash, haematuria or decreased urine output, dyspnoea or haemoptysis. If vasculitis is suspected, discontinue methimazole therapy and initiate appropriate intervention.

There have been post-marketing reports of acute pancreatitis in patients receiving carbimazole or its active metabolite, methimazole (thiamazole). In case of acute pancreatitis, methimazole should be discontinued immediately. Methimazole must not be given to patients with a history of acute pancreatitis after administration of carbimazole or its active metabolite methimazole (thiamazole). Re-exposure may result in recurrence of acute pancreatitis, with decreased time to onset.

Timazol contain lactose monohydrate. Patients with hereditary problems of galactose intolerance, the Lapp lactase deficiency or glucose-galactose malabsorption should not take this product.

Women of childbearing potential and pregnancy

Women of childbearing potential have to use effective contraceptive measures during treatment.

The use of methimazole in pregnant women must be based on the individual benefit/risk assessment. If methimazole is used during pregnancy, the lowest effective dose without additional administration of thyroid hormones should be administered. Close maternal, foetal and neonatal monitoring is warranted.

Effects on ability to drive and use machines

The effect on the ability to drive and use machines is not known.

210 mm

TOKUWA (P) (THAILAND)	Customer	เภสัชกรวรมศร์ประสิทธิ์	Size	150x210 mm	Confirm By
	Sales	ธำนิษฐ์	Color	1COLOR	
	Artist	ยศ	Job Name	(PIT0014) TIMAZOL LEAFLET	
	Date	28/11/67	Material	(MALAYSIA) (Rev:4)	
	Modify		Machine		
รับรองงานถูกต้องจึงพิมพ์ได้					

หลังเซ็นต์ Confirm กรุณาส่ง E-mail : yot5864@hotmail.com หรือ โทร 02-431-3158 กลับมาเพื่อยืนยันการผลิต

หน้า

DRUG INTERACTIONS

Concomitant Drugs	Interactions
Anticoagulants (oral)	Due to potential inhibition of vitamin K activity by methimazole, the activity of oral anticoagulants (e.g., warfarin) may be increased. Additional monitoring of PT/INR should be considered, especially before surgical procedures.
β-adrenergic blocking agents	Hyperthyroidism may cause an increased clearance of beta blockers with a high extraction ratio. A dose reduction of beta-adrenergic blockers may be needed when a hyperthyroid patient becomes euthyroid.
Digitalis glycosides	Serum digitalis levels may be increased when hyperthyroid patients on a stable digitalis glycoside regimen become euthyroid. A reduced dosage of digitalis glycosides may be needed.
Theophylline	Theophylline clearance may decrease when hyperthyroid patients on a stable theophylline regimen become euthyroid. A reduced dose of theophylline may be needed.

PREGNANCY & LACTATIONWomen of childbearing potential

Women of childbearing potential have to use effective contraceptive measures during treatment (see Warnings / Precautions).

Pregnancy

Methimazole crosses the placenta but, provided the mother's dose is within the standard range and her thyroid status is monitored; there is no evidence of neonatal thyroid abnormalities. Studies have shown that the incidence of congenital malformations is greater in the children of mothers whose hyperthyroidism has remained untreated than in those who have been treated with methimazole.

However, cases of congenital malformations have been observed following the use of methimazole during pregnancy. A causal relationship of these malformations, especially choanal atresia and aplasia cutis congenita (congenital scalp defects), to transplacental exposure to methimazole cannot be excluded.

Therefore, the use of methimazole in non-pregnant women of childbearing potential should be based on individual risk/benefit assessment (see Warnings / Precautions).

Cases of renal, skull, cardiovascular congenital defects, exomphalos, gastrointestinal malformation, umbilical malformation and duodenal atresia have also been reported. Therefore, methimazole should be used in pregnancy only when propylthiouracil is not suitable.

If methimazole is used in pregnancy, the dose must be regulated by the patient's clinical condition. The lowest dose possible should be used, and this can often be discontinued three or four weeks before term, in order to reduce the risk of neonatal complications. The blocking-replacement regimen should not be used during pregnancy since very little thyroxine crosses the placenta in the last trimester.

Hyperthyroidism in pregnant women should be adequately treated to prevent serious maternal and foetal complications.

Methimazole is able to cross the human placenta. Based on human experience from epidemiological studies and spontaneous reporting, methimazole is suspected to cause congenital malformations when administered during pregnancy, particularly in the first trimester of pregnancy and at high doses. Reported malformations include aplasia cutis congenita, craniofacial malformations (choanal atresia; facial dysmorphism), exomphalos, oesophageal atresia, omphalomesenteric duct anomaly, and ventricular septal defect.

Methimazole must only be administered during pregnancy after a strict individual benefit/risk assessment and only at the lowest effective dose without additional administration of thyroid hormones. If methimazole is used during pregnancy, close maternal, foetal and neonatal monitoring is recommended (see Warnings/Precautions).

Lactation

Methimazole is contraindicated in nursing mothers as the drug is excreted in breast milk.

ADVERSE REACTIONS / SIDE EFFECTS

Serious adverse reactions (which occur less frequently than the minor adverse reactions) include inhibition of myelopoiesis (agranulocytosis, granulocytopenia and thrombocytopenia), aplastic anaemia, drug fever, a lupus-like syndrome, insulin autoimmune syndrome (which can result in hypoglycaemic coma), hepatitis (jaundice may persist for several weeks after discontinuation of the drug), periarteritis and hypoproteinaemia. Nephritis and vasculitis occur very rarely. Cholestatic jaundice, fulminant hepatitis, encephalopathy, hepatic necrosis and death have been rarely reported (see Warnings/ Precautions).

There are reports of a vasculitis, often associated with the presence of anti-neutrophilic cytoplasmic antibodies (ANCA), resulting in severe complications (see Warnings / Precautions).

Minor adverse reactions include skin rash, urticaria, nausea, vomiting, epigastric distress, arthralgia, paraesthesia, loss of taste, abnormal loss of hair, myalgia, headache, pruritus, drowsiness, neuritis, oedema, vertigo, skin pigmentation, jaundice, sialadenopathy, anorexia, right upper-quadrant pain and lymphadenopathy.

Post-Market Adverse Reactions

The following adverse reactions have been reported from marketing experience with methimazole. Because these reactions are reported voluntarily from a population of uncertain size, it is not always possible to reliably estimate their frequency or establish a causal relationship to drug.

- Acute pancreatitis (*frequency: not known*) (see Warnings / Precautions)
- Vasculitis (*frequency: very rarely*) (see Warnings / Precautions)

Cases of congenital anomalies have been reported in neonates, whose mothers were treated with methimazole during pregnancy: aplasia cutis congenita, craniofacial malformations (choanal atresia; facial dysmorphism), exomphalos, oesophageal atresia, omphalomesenteric duct anomaly, and ventricular septal defect (see Warnings/Precautions, Pregnancy & Lactation).

SYMPTOMS & TREATMENT OF OVERDOSAGE

Symptoms may include nausea, vomiting, epigastric distress, headache, fever, joint pain, pruritus and oedema. Aplastic anaemia (pancytopenia) or agranulocytosis may be manifested in hours to days. Less frequent events are hepatitis, nephrotic syndrome, exfoliative dermatitis, neuropathies and CNS stimulation or depression. No information is available on the median lethal dose of the drug or the concentration of methimazole in biologic fluids associated with toxicity and/or death.

Treatment: The stomach should be emptied by inducing emesis or by gastric lavage. Absorption of drugs from the gastrointestinal tract may be decreased by giving activated charcoal, which is more effective than emesis or lavage. Repeated dose of charcoal may hasten elimination of drugs. The patient's airway should be safeguarded when employing gastric emptying or charcoal.

Forced diuresis, peritoneal dialysis, haemodialysis or charcoal haemoperfusion have not been established as beneficial for an overdose of methimazole.

STORAGE CONDITIONS

Do not store above 30°C.

Protect from light.

Keep out of reach of children.

Jauhi daripada kanak-kanak.

SHELF LIFE: Please refer to the outer box label.

PACK SIZE

Alu-White PVC blister of 10 tablets.

Box of 100 and 500 tablets

Manufactured by:

Sripasit Pharma Co., Ltd.

216 Moo 6 Suanluang, Krathum Baen,

Samut Sakhon 74110, Thailand.

Date of Revision: November 2024

PIT0014 Rev : 4

210mm

TODOKA (THAILAND)	Customer	เภสัชกรวรมศร์ประสิทธิ์	Size	150x210 mm	Confirm By
	Sales	ธาฬินันท์	Color	1COLOR	
	Artist	ยศ	Job Name	(PIT0014) TIMAZOL LEAFLET	
	Date	28/11/67	Material	(MALAYSIA) (Rev:4)	
	Modify		Machine		

หลังเซ็นต์ Confirm กรุณาส่ง E-mail : yot5864@hotmail.com หรือ โทร 02-431-3158 กลับมาเพื่อยืนยันการผลิต

หลัง